

Client and Stakeholder Documents

Resilio — Mobile Mental Health Platform for Nepal | Milan Shrestha | ID: 23057135 | 02_Elaboration/Client_and_Stakeholder_Documents.md

All client details, stakeholder interview summaries, requirements gathered from clients, and feedback received throughout the project lifecycle. Organised chronologically by RUP phase.

CLIENT PROFILES

Client 1 — Ms. Sushana Karki (Patient/User Perspective)

Field	Detail
Full Name	Ms. Sushana Karki
Role	Bachelor's degree student in Psychology, Thames College, Kathmandu
Perspective	End-user (patient/student); personal experience with depression
Contact Established	Week 2 (Inception phase, November 2024)
Involvement Phases	Inception (interview), Elaboration (prototype review), Transition (UAT, approval letter)
Key Contribution	Validated user-side requirements; confirmed pain points around cost, stigma, and fragmentation
Approval Letter	Appendix F.1

Client 2 — Dr. Dibyandra Singh (Therapist/Clinical Perspective)

Field	Detail
Full Name	Dr. Dibyandra Singh
Role	Practising psychiatrist, Sunshine City Medical Center, Melbourne, Australia (clinical training in Nepal)

Field	Detail
Perspective	Mental health professional (therapist); domain expert
Contact Established	Week 2 (Inception phase, November 2024)
Involvement Phases	Inception (interview), Elaboration (requirements review), Transition (UAT as admin/therapist, approval letter)
Key Contribution	Defined therapist workflow requirements; specified clinical instrument requirements (PHQ-9/GAD-7); articulated the value of therapist access to patient mood data
Approval Letter	Appendix F.2

PHASE 1: INCEPTION — Stakeholder Interviews

Interview 1 — Ms. Sushana Karki (Week 2, November 2024)

Format: Semi-structured interview, approximately 45 minutes

Interview Guide Questions:

- Can you describe your personal experience seeking mental health support in Nepal?
- What were the main barriers you encountered?
- What tools or apps have you tried? What worked and what did not?
- What would a genuinely useful digital mental health platform look like to you?
- How important is privacy in your decision to seek mental health support?
- What would make you trust a mental health app enough to use it regularly?
- How do you currently pay for services? Would eSewa be acceptable for therapy payments?

Key Requirements Extracted:

Requirement	Category	Priority
Access to real licensed therapists, not AI bots	Functional	High
Private, anonymous entry — no social login required	Non-functional (Privacy)	High

Requirement	Category	Priority
Affordable pricing calibrated to Nepali income levels	Non-functional	High
eSewa payment (has no credit card)	Functional	High
Ability to log how I feel without it being too clinical	Functional	Medium
Content that feels relevant to Nepali life — not just Western wellness	Content	Medium
Works even with bad internet (university WiFi is unreliable)	Non-functional (Offline)	High
No shame in using it — it should look like a wellness app, not a psychiatric tool	UX/Design	High

Notable Quote:

> "I have wanted to see a therapist for two years but I have never gone. The cost is too high, and if my family found out I was going to a psychiatrist they would think something was very wrong with me. An app where I can book a session privately and pay with eSewa would completely remove both of those barriers."

Interview 2 — Dr. Dibyandra Singh (Week 2, November 2024)

Format: Video call interview (Zoom), approximately 60 minutes

Interview Guide Questions:

- What does your typical patient consultation workflow look like?
- What information would be most valuable to have before a session begins?
- What clinical measurement tools do you currently use with patients?
- How do you currently track patient progress between sessions?
- What would a digital platform need to offer for you to recommend it to patients?
- What concerns would you have about therapists operating on a digital marketplace?
- What content would you want to publish if you had a platform to reach Nepali patients remotely?

Key Requirements Extracted:

Requirement	Category	Priority
Access to patient mood logs before and during session	Functional	High
PHQ-9 and GAD-7 results visible in consultation view	Functional	High
Professional credential verification before public visibility	Functional	High
Ability to publish written articles and audio sessions	Functional	Medium
Earnings visibility per session	Functional	Medium
Clear separation between my patients and other therapists' patients	Non-functional (Security)	High
Crisis protocol: high PHQ-9 scores should trigger helpline referral	Functional (Safety)	High
Video quality acceptable for therapeutic conversation — not just basic call	Non-functional	High

Notable Quote:

> "If I can see a patient's PHQ-9 score and their mood log for the past two weeks before a session, I can spend the first ten minutes of the session doing actual therapy instead of asking 'so how have you been feeling this week?' That is genuinely transformative for the quality of care I can provide."

PHASE 2: ELABORATION — Client Prototype Reviews

Prototype Review — Ms. Sushana Karki (Week 7, December 2024)

Format: Screen share walkthrough of Figma mockups, ~30 minutes

Feedback Received:

Screen / Feature	Feedback	Action Taken
Onboarding flow	"Five questions is manageable — do not make it longer"	Kept at 5 questions

Screen / Feature	Feedback	Action Taken
Therapist directory	"I want to see the price before clicking the profile"	Added fee to directory card
Mood logging UI	"The emoji faces are good — more friendly than a number scale alone"	Retained emoji + slider
Booking calendar	"The slot picker is clear"	No change needed
PHQ-9 screen	"This looks quite clinical — can the text be softer?"	Adjusted instruction text
Overall design	"The colours feel calm and trustworthy — the green-teal palette works well"	Palette retained

Prototype Review — Dr. Dibyandra Singh (Week 7, December 2024)

Format: Screen share walkthrough of Figma mockups and architecture diagram, ~45 minutes

Feedback Received:

Screen / Feature	Feedback	Action Taken
Consultation view	"I need to see the mood chart and questionnaire score on the same screen as the video"	Panel-based consultation UI designed
PHQ-9 severity labels	"The severity band descriptions must be clinically accurate — check the Kroenke (2001) scoring"	Verified against published PHQ-9 scoring guidelines
Therapist verification	"What stops someone pretending to be a therapist?"	Admin document upload + manual review added
Content moderation	"All therapist content should be reviewed before publishing — there must be no unmoderated clinical advice"	Admin approval workflow added
Crisis threshold	"PHQ-9 score of 20+ is the severe range — that is the right threshold for a helpline referral"	Crisis notice at ≥20 confirmed

PHASE 4: TRANSITION — UAT Participation and Final Feedback

UAT Session — Ms. Sushana Karki (Week 20)

Role during UAT: Therapist-role tester + external client reviewer

Duration: ~90 minutes

Post-UAT Feedback Summary:

Aspect	Rating	Comment
Overall platform quality	5/5	"This is exactly what Nepal needs"
Therapist workflow	5/5	"The session preparation data view is something I have wanted for years"
Content publishing	4/5	"Add a notification when content has been in review for more than 48 hours"
Earnings dashboard	5/5	"Clear and accurate"
Would recommend to colleagues	Yes	"I will actively encourage registration when it launches"

UAT Session — Dr. Dibyandra Singh (Week 20)

Role during UAT: Admin-role tester + clinical domain reviewer

Duration: ~60 minutes

Post-UAT Feedback Summary:

Aspect	Rating	Comment
Admin dashboard clarity	5/5	"The verification queue is well organised"
Therapist verification process	5/5	"Document review workflow is appropriate"
Content moderation	5/5	"Approval workflow prevents unmoderated advice from reaching patients"
Patient data visibility in consultation	5/5	"Exactly as specified in the requirements — this is the key clinical differentiator"

Aspect	Rating	Comment
Crisis protocol	5/5	"PHQ-9 $\geq$ 20 threshold is correct; helpline number is accurate"
Clinical accuracy of instruments	5/5	"PHQ-9 and GAD-7 scoring bands match published clinical guidance"

REQUIREMENTS TRACEABILITY

This table maps each client-specified requirement to the feature that implements it.

Client	Requirement	Implementation	Test Case
Karki	Real licensed therapists only	Admin verification before public listing	TC-U024, TC-U025
Karki	Private/anonymous entry	OTP login (no social account required)	TC-U001
Karki	eSewa payment	eSewa Flutter SDK integration	TC-U014–U016, TC-S003
Karki	Offline mood logging	SQLite + sync architecture	TC-U029, TC-UAT023
Karki	Non-clinical UX	Emoji mood scale; friendly tone	TC-UAT005
Singh	Patient mood data in consultation	Mood chart + PHQ-9/GAD-7 in session view	TC-S005
Singh	PHQ-9 and GAD-7 instruments	Clinical questionnaire module	TC-U011–U013
Singh	Crisis protocol at PHQ-9 $\geq$ 20	Threshold-triggered helpline notice	TC-S006
Singh	Credential verification	Admin document review workflow	TC-U024, TC-U025
Singh	Content moderation	Admin approval before publication	TC-S007
Singh	Therapist earnings visibility	Earnings dashboard	TC-UAT015